

# Faithful Families, Thriving Communities: Bridging Faith and Health Through a State-Level Partnership

Faith communities are increasingly being recognized as important leaders in community health work, both in the United States and globally. However, faith communities are also often working at or beyond capacity in terms of meeting the social, emotional, and health needs of the communities they serve.

To bridge these gaps and build on the significant assets that faith communities possess, the Faithful Families Thriving Communities program was created in 2007 as a partnership between the North Carolina Division of Public Health and North Carolina State University. Based on the social-ecological model, Faithful Families trains and empowers lay leaders from the faith community to coteach and partner with health educators from Cooperative Extension or public health as they support individual members and the faith community at large to adopt changes that can address inequities in their communities related to access to healthy foods and places to be active.

This essay highlights the formation and development of this partnership, outlining challenges and best practices for this kind of “bridging” between public health and communities of faith, particularly related to state-level programmatic support and collaboration. (*Am J Public Health*. 2019;109:363–368. doi:10.2105/AJPH.2018.304869)

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See also Cutts and Gunderson, p. 361.

**P**ublic health researchers and practitioners have recognized religious communities as vital contributors and leaders in community health work, noting the intrinsic assets they possess that can and do contribute to human health and flourishing.<sup>1–3</sup> In the United States in particular, African American churches and religious organizations have attempted to mitigate the impacts of centuries of structural racism and abuse (at times at the hands of the medical and public health community),<sup>4,5</sup> by serving as a place of refuge and first line of defense for health, healing, and social services.<sup>6,7</sup> However, as Wuthnow and others have pointed out, faith communities are often working at or beyond capacity when it comes to their ability to meet the social and emotional needs of their members and the broader community.<sup>8</sup> In addition, as researchers have noted, the ways that public health researchers and practitioners have engaged religious communities and practices have not always been well-defined. Given this, there is an increased need for dialogue about what it means to do faith-based health promotion work.

This article outlines key findings from a state-level faith-based health promotion program, Faithful Families Thriving Communities (Faithful

Families), which was designed to acknowledge and harness the inherent wisdom and indigenous assets that faith communities possess to address the complex factors that shape religious life and communities. Beginning with an overview of the program’s founding and initial pilot testing, this article articulates challenges, best practices, and lessons learned during the 10 years of program implementation. The program has leveraged state and local partnerships in North Carolina—specifically between Cooperative Extension (Extension) and public health—to bridge these gaps and support faith communities as they seek to connect faith and health for their members and the community at large. This article argues that faith-based health promotion programs should honor the profound assets that faith communities possess while also acknowledging the lack of access to social, community, and public health resources that many low-income communities, rural communities, or communities of color face.<sup>9</sup>

## BACKGROUND AND HISTORY OF FAITHFUL FAMILIES

In the early 2000s, innovative programming in religion and health was taking place across North Carolina, drawing on a long history of faith-based health promotion in the state that began as early as 1978, through the General Baptist State Convention’s Human Services Project.<sup>10</sup> This project engaged lay health advisors to provide screenings, referrals, and information about healthy behaviors to more than 1700 Black churches throughout North Carolina.<sup>11</sup> It created a model for collaboration among academic institutions, faith communities, and public health that emphasized the need for a “shared vision,” strong support from clergy, and the key role that lay leaders can play in faith-based health promotion.<sup>10</sup> In 2008, Duke Divinity School began their 10-year Duke Clergy Health Initiative, which surveyed United Methodist clergy across the state about their health behaviors and practices.<sup>12,13</sup> In the same year, the North Carolina

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Council of Churches launched a faith-based health initiative that would come to be called Partners in Health and Wholeness. In addition, in 2012, Wake Forest Baptist Medical Center brought the “Memphis Model” of congregational-hospital partnership to the state, hiring Gary Gunderson to lead what was called the Faith-Health program. The Memphis Model connects hospital patients with trained faith community liaisons via their electronic medical records, providing the patient with postdischarge support from a trained caregiver in their congregation.<sup>14</sup> The FaithHealth team adapted the model for North Carolina, now called the “North Carolina Way,” which includes a broad network of connectors, clergy, fellows, and liaisons who promote health and provide care in local communities.

Building on this growing momentum in faith and health in North Carolina, in 2007, the North Carolina Division of Public Health and North Carolina State University developed the Faithful Families program, which harnessed the state-level resources these 2 organizations offered for faith-based health promotion. This included wide networks of health educators across local health departments (LHDs) and Extension offices, broad state-level expertise in supporting systems-level changes to promote health in communities, well-tested programs and materials to promote healthy eating and physical activity, and the unique partnership that these organizations shared at the state and local levels, including leadership of the state-level Eat Smart, Move More North Carolina movement.<sup>15</sup> To begin this work, these 2 organizations convened more than 40 state and local stakeholders from religious communities and health organizations to solicit feedback on faith-based

health promotion programming, priorities, and design. Participants at the meeting agreed that the program should (1) address health disparities and focus on serving minority and low-income communities, (2) not include scripture or references to any particular religious tradition and instead be adaptable and open to any religious organization that wants to participate, and (3) not only focus on individual behaviors but also address social and community-level changes.<sup>16</sup> Many members of the stakeholder group served on a volunteer advisory committee, which advised the project throughout its 3-year pilot phase.

Faithful Families is based on the social-ecological model of behavior change (Table 1) and includes a 9-week educational series that helps faith community members to learn best practices in nutrition and physical activity, connect their faith and health through discussion questions and activities, and explore community-level changes that their faith community can make to support health (see the box on page 367). On the basis of research that demonstrated the importance of lay leadership and knowledge,<sup>3,17</sup> lay leaders (non-clergy members) from the faith community are trained to coteach Faithful Families classes alongside health educators from Extension or an LHD. Lay leaders are essential to program delivery and help participants to connect faith and health by integrating religious practices, beliefs, or scriptures from their traditions into each lesson. These religious practices and discussions are unique to each community and have included activities such as labyrinth walking in a Christian church, connecting embodied practices of daily Muslim prayer to the need for physical activity or movement each day, and

discussions about food insecurity and the foods that are available in faith-based food pantries. During each class, lay leaders facilitate discussions that lead to concrete planning about the policy, system, and environmental (PSE) changes the faith community could implement to improve access to healthy foods and safe places to be active in their faith community and the community at large. For example, an African American church started an on-site farmers’ market to address the widening gap between the healthy, local foods that were available in farmers’ markets in wealthy neighborhoods and the lack of availability in neighborhoods affected by systemic poverty. Other faith communities have opened up their gym or playground facilities beyond their membership, providing access to safe, affordable places to play in their neighborhoods.<sup>18</sup>

In North Carolina, Faithful Families is implemented through a state network of health educators from LHDs and county Extension offices who possess a body of resources and connections available through the state’s public health and Extension systems. By working with public health educators, faith communities have mapped out walking trails, installed signage on site to promote physical activity, and connected members with diabetes prevention programs. In partnership with Extension educators, faith communities have connected with or started local farmers’ markets, hosted food preservation classes, and started or refurbished community gardens.

## PILOT PROJECT

Through initial funding from the Kate B. Reynolds Charitable Trust, Faithful Families was piloted in 41 Christian churches in 4 counties over a 3-year period

(2008–2011). Faith communities were recruited by local Extension staff, who used Census data to focus on neighborhoods with higher levels of poverty in the county, as initial program implementation was through the Expanded Food and Nutrition Education Program (EFNEP), which works exclusively with low-income families. Within these congregations, Extension staff worked closely with clergy and lay leaders to distribute printed copies of member health assessments during services or regular gatherings. A total of 941 respondents at participating faith communities completed the assessments, which revealed that 62.6% of respondents were living in poverty, as designated by the 2011 Department of Health and Human Services federal poverty guidelines. In addition, 7.1% respondents reported that they suffered from heart disease, 24.3% from high cholesterol, 20.4% from arthritis, 37.1% from diabetes, and 37.1% from high blood pressure.<sup>9</sup> This was a convenience sample, which used paper surveys distributed during worship times, so is not representative of the entire congregation; however, it does offer some insights as to the health factors individuals faced in the pilot project.

The program forged close partnerships between local health educators and faith communities and provided an avenue for participants to share and grow their perceptions about the connections between food and health. In the pilot, participants were emphatic that their congregation’s foods were integral to their sense of community and identity.<sup>19</sup> These foods, they affirmed, should not be discarded or rejected; at the same time, members also recognized that people in their congregations were suffering from preventable chronic diseases that could be

**TABLE 1—Faithful Families Program Across the Social-Ecological Model: North Carolina, 2018**

| Level                | Program Approach                                                                                                                                                                                                                                                                                                                                                             | Program Outcomes                                                                                                                                                                                                                                                                                                                                         |
|----------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Community            | Program facilitators help faith communities extend their reach into the local community by connecting faith communities in local coalitions and organizations implementing community-wide health initiatives.                                                                                                                                                                | Faith communities have joined local coalitions working to increase access to healthy food and safe places for physical activity, opened up faith community spaces for physical activity in the local community, and established on-site farmers markets to increase access to local food.                                                                |
| Organizational       | Each faith community completes the Faith Community Assessment to evaluate their community's needs and available assets for policy, systems, and environmental (PSE) changes that support healthy eating and physical activity. Facilitators provide the community with resources and ongoing technical assistance for implementation of PSE changes in the faith community.  | Faith communities have established policies to serve water and unsweetened beverages at all events, repaved parking lots to serve as a walking track, established guidelines that all meals offered at the faith community must follow healthy food guidelines, established community gardens, and integrated physical activity breaks for all meetings. |
| Social/Interpersonal | Through group discussions and goal setting, individuals and families encourage each other to change risk behaviors and set goals for healthy eating and daily physical activity.                                                                                                                                                                                             | Faith communities have participated in walking challenges and established walking clubs to encourage members to increase daily physical activity. They have also established recipe exchanges to share healthy recipes.                                                                                                                                  |
| Individual           | Trained facilitators and lay leaders work with each faith community to deliver a 9-lesson curriculum on healthy eating and physical activity strategies. The lessons are discussion-based and include recipe taste tests, physical activity breaks, and goal setting for healthy behavioral changes. Behavioral changes are evaluated with participant entry and exit forms. | According to 2017 participant entry and exit data, 78% of participants now practice better food resource management, 93% have improved dietary intake, 49% have improved their daily physical activity, and 67% have improved their food safety practices.                                                                                               |

addressed through adaptations of recipes, such as baking instead of frying. Similarly, participants in the pilot voiced their appreciation that the program encouraged making changes to support health in their everyday lives rather than emphasizing weight loss or body size. As one lay leader told us in a focus group, “Because a lot of them they realize that they are a little overweight, but to stress on losing weight, it just stresses them out even more . . . they were so tired of programs geared towards losing weight.” This echoes findings about the role weight stigma plays in hindering weight loss and the need to focus on the “psychological meaning” people ascribe to their bodies and health.<sup>20</sup> During the pilot project, faith communities implemented more than 140 PSE changes to support health in their communities, which in turn impacted more than 9000 members of these congregations.

These changes included creating policies to serve water or offer healthier food options at meals, planting community gardens, marking out parking lots for walking routes, and making physical activity a regular part of meetings or services.<sup>9,21</sup>

Participants in the 9-week series of classes completed EFNEP entry and exit forms at the first and last class. Of the 566 individuals who completed the classes, 83% reported positive changes in nutrition behavior, such as eating more fruits and vegetables or reading nutrition labels.<sup>9</sup> In addition, 11 focus groups were conducted between 2008 and 2010, 6 with participants and 5 with lay leaders. These 61 focus group participants revealed that lay leaders played a crucial role in implementing PSE changes in each faith community, in addition to helping participants to think critically about how their physical and spiritual health were

connected.<sup>21</sup> The results from the pilot led to Faithful Families becoming recognized as a practice-tested intervention from the University of North Carolina Chapel Hill Center for Training and Research Translation and widespread adoption in North Carolina through Extension (Family and Consumer Science agents), EFNEP, and LHDs.

In 2014, Faithful Families state staff began training extension and public health organizations outside North Carolina to implement the program and has since developed a yearly national training, quarterly calls and webinars, and regular newsletters for the 21 national partners who now implement the program across the country.<sup>22</sup> Faithful Families was accepted as a Supplemental Nutrition Assistance Program Education (SNAP-Ed) program nationally in 2016. In addition, the program was selected as the signature program

for the US Department of Agriculture–funded Regional Center of Excellence in Nutrition Education and Obesity Prevention, leading to its implementation by the Universities of Arkansas, Tennessee, and Florida through their EFNEP and SNAP-Ed programs from 2015 to 2016. Similar results and lessons learned were reported across the 3 states.

## CHALLENGES AND CONFLICTS

As others have reflected, any attempt to develop partnerships with religious communities does not come without struggle, challenge, and even conflict.<sup>3,23–25</sup> Faithful Families has experienced significant barriers around data collection, program fidelity, and readiness. Such barriers provide an opportunity to explore the ways in which state and local partnerships can help similar

programs to grow and evolve within the boundaries of evidence to better address distinct faith community needs and assets.

One of the program's earliest challenges centered on data collection. In the 3-year program pilot, all members of participating faith communities were asked to complete a member health and interest survey. The rationale was to ensure that the program was meeting the requests of funders and stakeholders in serving communities that experience health disparities, to help faith communities have a better sense of the chronic conditions that members of their congregation were facing, and to garner a sense of the types of programs, changes, and environmental supports that faith community members might be most interested in pursuing. Extension staff faced resistance to these surveys, which asked people to provide their income (on a scale) and sensitive health information (diagnoses of cancer, diabetes, heart disease, health behaviors). In the pilot project, only 10.5% of congregational membership completed surveys. These types of surveys placed great burden on the health educators, as they had to find the best way to publicize, distribute, and collect these forms among hundreds of faith community members.

Faithful Families discontinued that survey and now uses a Faith Community Assessment, which is completed by the lay leader or clergy member, to learn about the faith community's basic demographics, religious affiliations, leadership structure, physical location, and any policies, systems, or practices that could be harnessed to promote health. In addition, participants complete entry and exit forms at the first and last class, which track changes in eating and physical activity practices. Through the years,

these outcomes have been consistent<sup>22</sup>; most participants show a positive change in 1 or more nutrition practices after the class series ends; for example, among the 197 participants who completed classes in 2017, 93% showed improvement in 1 or more nutrition practices, such as eating more fruits and vegetables or planning meals ahead of time.<sup>9,26</sup>

As the program has grown and expanded across the United States, it has been a challenge to ensure that the program is being delivered as it has been developed. Despite the emphasis on the social-ecological model, some faith communities and facilitators adopt 1 element of the program and not the other. For some faith communities, scheduling issues have made it impractical for them to commit to 9 weeks of classes. Although some are ready to implement a policy change within their congregation, such as serving water at all events, it is more difficult for a congregation with fewer resources (e.g., staffing or space) to tackle broader changes for health, such as opening up their spaces for the community to use for physical activity. Many of the faith communities that participated in the program experienced challenges in terms of lack of funding to support community efforts, but local program facilitators have connected them with North Carolina Council of Churches' Partners in Health and Wholeness program, which funds minigrants to implement and sustain efforts for healthy eating and active living.

Building strong relationships with faith communities takes time, patience, and a respect for the congregation's priorities. The Faithful Families partnership model aims to support program facilitators as they engage faith

communities by meeting the congregation at their stage of readiness and providing resources and tools that harness their assets and meet their particular needs. For instance, with faith communities that have not been interested or ready to implement Faithful Families, health educators have built relationships over the long term by connecting faith community members to local resources that address their concerns or priorities. Extension agents have provided technical assistance on gardens and growing, links to diabetes prevention programs, and tools to hold on-site health fairs. Drawing on networks and programming offered through North Carolina Division of Public Health and North Carolina Extension, Faithful Families has developed supplemental activities and programs to engage faith communities, such as the Faithful Families Walking Challenge and Farmers' Market Tours.

Similarly, some program facilitators have struggled with the long-term engagement that is a cornerstone of the program's sustainability. Unlike short-term health-promotion programs, Faithful Families encourages facilitators to enter into a long-term partnership with a faith community, to walk alongside them as they address the health issues in their community, and support that faith community in becoming a catalyst for change. To address some of the challenges related to long-term engagement, Faithful Families has adapted facilitator trainings to share strategies for identifying and engaging county-level partners that could support these efforts. In addition, because initial program implementation was carried out in Christian churches, state program staff have developed partnerships with a

local mosque and carried out a 3-year implementation of the program with a diverse group of more than 50 refugee and immigrant women.

Lastly, Faithful Families has grown quickly in the past few years. However, because of shifts in state spending priorities, LHDs and Extension offices have experienced cuts to funding and staff. This creates challenges in counties where there are not enough health educators trained to implement the program and where requests from local faith communities outweigh the program's capacity to provide the program.

## BEST PRACTICES

Looking back at the 10 years of development and implementation of Faithful Families offers a unique insight on best practices. First, Faithful Families emphasizes listening. The program started with stakeholder meetings and a state-wide advisory board, which has grown into a network of national and state partners who participate in quarterly webinars and calls to discuss program implementation, direction, and needs. At the local level, the lay leaders not only coteach the lessons, but they also guide the changes that are made in their faith communities to support health, ensuring that it is the faith community itself that drives the work. The program, which was originally named Faithful Families Eating Smart and Moving More, began with a focus on healthy eating and physical activity; however, the program is currently engaging in a strategic planning process with state and local partners to better address the broad and systemic issues that are facing faith communities today. The new name



## FAITHFUL FAMILIES PROGRAM LESSON OVERVIEW

| Week Covered | Lesson Name                              |
|--------------|------------------------------------------|
| 1            | Plan: Know What's for Dinner             |
| 2            | Shop: Get the Best for Less              |
| 3            | Shop for Value, Check the Facts          |
| 4            | Fix It Fast, Eat at Home                 |
| 5            | Choosing More Fruits and Vegetables      |
| 6            | Fix It Safe                              |
| 7            | Making Smart Drink Choices               |
| 8            | Choosing to Move More Throughout the Day |
| 9            | Making the Connection                    |

Faithful Families Thriving Communities reflects this shift toward broader issues of community development and social justice, in addition to mental health and stress, all of which have been identified by faith communities as vital to their continued health and well-being. Listening—to stakeholders, faith communities, and national and local partners—has been crucial in ensuring the long-lasting impacts of this program.

Second, Faithful Families has focused on building the capacity of local leaders in faith communities, empowering them to become health ambassadors in their faith community and the community at large.<sup>21</sup> Initial stakeholders affirmed that faith-based health-promotion programs should honor the wisdom in each faith community and support lay people and clergy as they determine ways to integrate faith and health for members and their community. Building the capacity of these local stakeholders is essential because they know best the assets, resources, and issues that their communities face and possess. The discussion questions in each Faithful Families class encourage people to talk about how they could support

health in their communities, creating a dedicated group of health ambassadors to champion changes and encourage their fellow community members to embrace new health practices. As research on social capital demonstrates, this can lead to inward or outwardly focused social programs in religious organizations.<sup>27</sup> For some of these faith communities, this has meant adopting internal changes to improve the health of their members, such as adopting healthy eating policies or hosting fitness events. However, for some of these faith communities, the desire has been to reach outward, into the broader community, through practices such as opening up their facilities for physical activity (“shared use”) or planting community gardens where produce is donated to a local food pantry.<sup>18</sup> Building the capacity of these lay leaders and dedicated faith community members has been crucial to the program’s long-term sustainability and community impacts. The program continues to grow connections between these communities through biannual summits, as well as quarterly webinars, newsletters, and calls with national partners.

Third, partnership—at the state and local level—has been foundational to this work. Faithful Families state staff and local facilitators have leveraged and developed both long-standing and emergent partnerships to better engage faith communities across the state. For example, through funding from the Centers for Disease Control and Prevention (CDC), North Carolina Division of Public Health-funded regional coordinators supported broad-scale PSE changes through Faithful Families implementation in 5 regions (50 counties) across the state from 2014 to 2018. These regional health department staff worked closely with Extension agents and program assistants in their counties, combining strengths of both organizations to support faith communities as they implemented changes at the individual, family, community, and county levels. This type of multilevel, community-based approach has ensured that the significant resources of both public health and Extension were being fully leveraged. The program’s aim has been to foster and sustain “bridging social capital” in the communities where facilitators have worked, forging connections between faith communities and the broader network of public health and social service organizations in their communities.<sup>28</sup> For example, Extension agents and EFNEP program assistants have strong skills in program delivery and nutrition education and a wealth of county-level resources for growing and supporting fresh, local foods. Through the Division of Public Health’s CDC funding, regional public health coordinators were able to provide significant assets related to systems-level changes to support healthy eating and physical

activity, leading to local partnerships between Extension and public health that mirrored strong state-level collaborations.

Finally, the Faithful Families program model emphasizes and encourages long-term engagement with the whole community—not just the faith communities—where facilitators work. Grants and programs often come and go, leaving communities frustrated with public health researchers and practitioners and generating distrust and ill will among the communities served. Faith-based health promotion programs, in particular, must adopt a long-term mindset, committing to engaging religious communities beyond the implementation of merely a program or a grant.<sup>29</sup> This has been one of the most significant lessons learned, and one that has the potential to shift practice for local educators and programmatic staff who are often used to delivering a program or project and then moving on to a new focus area. The next iteration of the program will focus on enhancing trainings to empower lay leaders as they initiate, implement, and sustain initiatives for healthy eating and active living in their faith communities and the community at large.

## CONCLUSIONS

As Idler and others have noted, religion is an often-ignored social determinant of health. Religion is a part of the social world for millions of people across the world, and religious communities have concrete impacts—for good and for ill—on the health and well-being of their members and broader communities.<sup>30</sup> However, public health researchers and practitioners have at times instrumentalized religious communities, seeing them as landing

sites for health promotion programming instead of living, breathing communities of complex and nuanced human beings.<sup>29,31,32</sup> As public health research and practice shifts from faith-placed (seeing faith communities as landing sites for health promotion interventions) to faith-based (partnering with faith communities to align health promotion with the religious organization's values, practices, and beliefs),<sup>24</sup> religion and health researchers need to be clear on just what faith-based health promotion work entails. As Cochrane et al. note,

[T]o treat religious constituencies mainly as an avenue for disseminating health messages, for example, underplays the importance of understanding them in their own terms, and of considering the way that their norms and values impact health delivery. It also undermines their possible role as partners in health with a contribution to make that goes beyond service delivery per se.<sup>29(p.348)</sup>

To that end, Faithful Families seeks to engage religious communities in identifying the health-promoting assets they possess while also connecting them to public health resources and programming that could strengthen and expand their reach into the broader community. Faithful Families' work centers on this question: how can public health researchers and practitioners develop long-standing relationships with faith communities that are built on trust, long-term engagement, and mutual respect? This work does not come without its particular conflicts and challenges, which researchers and practitioners should take into account when they engage in faith-based work. Despite these challenges, the Faithful Families program

will continue to build bridges between public health, Extension, and faith communities, to leverage these assets and help faith communities become change agents to support health and well-being for all communities. **AJPB**

## CONTRIBUTORS

Both authors contributed to the writing and editing of this article and serve as program leaders for the Faithful Families program.

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## CONFLICTS OF INTEREST

There are no conflicts of interest to disclose.

## HUMAN PARTICIPANT PROTECTION

The Faithful Families program evaluation has been approved by the North Carolina State University institutional review board.

## REFERENCES

- Oman D, ed. *Why Religion and Spirituality Matter for Public Health Evidence, Implications, and Resources*. Cham, Switzerland: Springer International Publishing; 2018.
- Idler E, ed. *Religion as a Social Determinant of Public Health*. Oxford, England: Oxford University Press; 2014.
- Campbell MK, Hudson MA, Resnicow K, Blakeney N, Paxton A, Baskin M. Church-based health promotion interventions: evidence and lessons learned. *Annu Rev Public Health*. 2007;28(1): 213–234.
- Jee-Lyn García J, Sharif MZ. Black Lives Matter: a commentary on racism and public health. *Am J Public Health*. 2015; 105(8):e27–e30.
- Gamble VN. Under the shadow of Tuskegee: African Americans and health care. *Am J Public Health*. 1997;87(11): 1773–1778.

- Mitchem S. *African American Folk Healing*. New York, NY: New York University Press; 2007.
- Mitchem S. “Jesus is my doctor”: healing and religion in African American women's lives. In: Barnes L, Sered S, eds. *Religion and Healing in America*. Oxford, England: Oxford University Press; 2005: 281–290.
- Wuthnow R. *Saving America?: Faith-Based Services and the Future of Civil Society*. Princeton, NJ: Princeton University Press; 2004.
- Hardison-Moody A, Dunn C, Hall D, Jones L, Newkirk J, Thomas C. Multi-level partnerships support a comprehensive faith-based health promotion program. *J Ext*. 2011;49(6). Available at: <https://www.joe.org/joe/2011december/iw5.php>. Accessed January 9, 2017.
- Holmes MAP, Hatch J. Health promotion and African-American Baptist churches in North Carolina. *N C Med J*. 2007;68(5):376.
- Braithwaite R, Taylor SE, Austin J. *Building Health Coalitions in the Black Community*. Thousand Oaks, CA: Sage Publications; 2000.
- Proeschold-Bell RJ, LeGrand S, Wallace A, et al. Tailoring health programming to clergy: findings from a study of United Methodist clergy in North Carolina. *J Prev Interv Community*. 2012; 40(3):246–261.
- Proeschold-Bell RJ, McDevitt PJ. An overview of the history and current status of clergy health. *J Prev Interv Community*. 2012;40(3):177–179.
- Cutts T, Gunderson G, Carter D, et al. From the Memphis Model to the North Carolina Way: lessons learned from emerging health system and faith community partnerships. *N C Med J*. 2017; 78(4):267–272.
- Dunn C, Kolasa KM. Development of a movement and state plan for obesity prevention, Eat Smart, Move More North Carolina. *J Nutr Educ Behav*. 2013;45(6): 690–695.
- Institute of Medicine. *Accelerating Progress in Obesity Prevention: Solving the Weight of the Nation*. Washington, DC: National Academies Press; 2012.
- Duff JF, Buckingham WW. Strengthening of partnerships between the public sector and faith-based groups. *Lancet*. 2015;386(10005):1786–1794.
- Hardison-Moody A, Edwards MB, Bocarro JN, et al. Shared use of physical activity facilities among North Carolina faith communities, 2013. *Prev Chronic Dis*. 2017;14:E11.
- Dodson JE, Gilkes CT. “There's nothing like church food”: food and the U.S. Afro-Christian tradition: remembering community and feeding the embodied S/spirit(s). *J Am Acad Relig*. 1995;63(3):519–538.
- Puhl RM, Quinn DM, Weisz BM, Suh YJ. The role of stigma in weight loss maintenance among U.S. adults. *Ann Behav Med*. 2017;51(5):754–763.
- Hardison-Moody A, Dunn C, Hall D, Jones L, Newkirk J, Thomas C. Faithful Families Eating Smart and Moving More: the role of lay leaders in the implementation of a faith-based health promotion program. *Int J Volunt Adm*. 2011; 27(2):18–28.
- Faithful Families Thriving Communities. Available at: <https://faithfulfamilies.com>. Accessed October 1, 2018.
- Tagai EK, Scheirer MA, Santos SLZ, et al. Assessing capacity of faith-based organizations for health promotion activities. *Health Promot Pract*. 2018;19(5): 714–723.
- Padela AI, Malik S, Vu M, Quinn M, Peek M. Developing religiously-tailored health messages for behavioral change: introducing the reframe, reprioritize, and reform (“3R”) model. *Soc Sci Med*. 2018; 204:92–99.
- Levin J. Faith-based initiatives in health promotion: history, challenges, and current partnerships. *Am J Health Promot*. 2014;28(3):139–141.
- Faithful Families Thriving Communities. 2017 North Carolina Annual Report. Available at: <https://faithfulfamilies.com/who-we-are/impact>. Accessed January 11, 2019.
- Beyerlein K, Hipp JR. Social capital, too much of a good thing? American religious traditions and community crime. *Soc Forces*. 2005;84(2):995–1013.
- Agnitsch K, Flora J, Ryan V. Bonding and bridging social capital: the interactive effects on community action. *Community Dev (Columb)*. 2006;37(1):36–51.
- Cochrane JR, McFarland D, Gunderson GR. Mapping religious resources for health: the African Religious Health Assets Programme. In: Idler E, ed. *Religion as a Social Determinant of Public Health*. Oxford, England: Oxford University Press; 2014:344–364.
- Idler E. Religion: the invisible social determinant. In: *Religion as a Social Determinant of Public Health*. Oxford, England: Oxford University Press; 2014:1–23.
- Hardison-Moody A. *When Religion Matters: Practicing Healing in the Aftermath of the Liberian Civil War*. Eugene, OR: Wipf and Stock; 2016.
- Barnes L. New geographies of religion and healing: states of the field. *Practical Matters J*. 2011;(4). Available at: <http://practicalmattersjournal.org/2011/03/01/new-geographies-of-religion-and-healing>. Accessed June 15, 2018.